Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0003	**Age / Sex**	79 / M
Setting	Emergency Dept	**Primary dx**	Acute-on-chronic SDH

Chief complaint

Confusion and drowsiness after a fall at home, found by daughter.

History of present illness

Fell from standing height 2 days ago and struck the left side of the head; did not seek care. Over 24 h became increasingly drowsy and incontinent. Daughter reports he spoke only a few words today.

Past history / medications

Atrial fibrillation on apixaban 5 mg twice daily (last dose this morning), hypertension, type 2 diabetes. Mild cognitive impairment at baseline but independent.

Examination

BP 164/88, HR 88 irregular, afebrile. Left frontal bruise. Pupils equal, reactive. Mild right arm drift. Speaks only isolated inappropriate words.

Glasgow Coma Scale

Eyes open to voice only. Verbal output limited to inappropriate words. Motor: localizes pain but does not follow commands consistently at first assessment.

GCS on arrival	Score	Response
Eye opening (E)	3	To voice
Verbal (V)	3	3 - Inappropriate words
Motor (M)	5	Localizes pain
TOTAL GCS		**11**

GCS at 3 h (after reversal)	Score	Response
Eye opening (E)	3	To voice
Verbal (V)	4	4 - Confused
Motor (M)	6	Obeys commands
TOTAL GCS		**13**

Investigations

CT head: left fronto-parietal crescentic subdural hematoma, 14 mm, with mixed density (acute on chronic), midline shift 7 mm. INR 1.2, anti-Xa level elevated. Glucose 9.1 mmol/L.

Assessment

Moderate traumatic brain injury (GCS 11) from acute-on-chronic subdural hematoma in an anticoagulated patient.

Plan / disposition

Anticoagulant reversal with andexanet alfa per protocol. Neurosurgical review: burr-hole drainage within 12 h. Repeat CT at 6 h. GCS improved to 13 after reversal. Admitted to neurosurgical high dependency unit.